1979 – HICKEY – TOTAL: 8 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸ **X**
- b) study controls for any additional factor (ALL factors) ✸
  - Reports pre-NST management and NST composition y
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) y
  - Reports setting (ICU or non-ICU) n
  - Reports demographics (age, gender) y

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸
- b) record linkage ✸ **X**
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

1980 – NEHME – TOTAL: 8 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸ **X**
- b) study controls for any additional factor (ALL factors) ✸
  - Reports pre-NST management and NST composition y
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) y
  - Reports setting (ICU or non-ICU) n
  - Reports demographics (age, gender) y

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸ **X**
- b) record linkage ✸
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

1984 – JACOBS – TOTAL: 8 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸ **X**
- b) study controls for any additional factor (ALL factors) ✸
  - Reports pre-NST management and NST composition y
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) y
  - Reports setting (ICU or non-ICU) n
  - Reports demographics (age, gender) y

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸
- b) record linkage ✸ **X**
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

1986 – TRAEGER – TOTAL: 8 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸ **X**
- b) study controls for any additional factor (ALL factors) ✸
  - Reports pre-NST management and NST composition y
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) y
  - Reports setting (ICU or non-ICU) n
  - Reports demographics (age, gender) y

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸
- b) record linkage ✸ **X**
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

1986 – FAUBION – TOTAL: 8 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ (= Silva et al, 1976) **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸ **X**
- b) study controls for any additional factor (ALL factors) ✸
  - Reports pre-NST management and NST composition — n
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) — y
  - Reports setting (ICU or non-ICU) — n
  - Reports demographics (age, gender) — n

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸
- b) record linkage ✸ **X**
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

1991 – OAKES – TOTAL: 7 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸
- b) study controls for any additional factor (ALL factors) ✸
  - Reports pre-NST management and NST composition — n
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) — y
  - Reports setting (ICU or non-ICU) — y
  - Reports demographics (age, gender) — y

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸
- b) record linkage ✸ **X**
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

1994 – GALES – TOTAL: 7 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸
- b) study controls for any additional factor (ALL factors) ✸
  - Reports pre-NST management and NST composition y
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) y
  - Reports setting (ICU or non-ICU) n
  - Reports demographics (age, gender) y

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸ **X**
- b) record linkage ✸
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

1996 – FISHER – TOTAL: 8 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸ **X**
- b) study controls for any additional factor (ALL factors) ✸
  - Reports pre-NST management and NST composition y
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) y
  - Reports setting (ICU or non-ICU) n
  - Reports demographics (age, gender) n

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸
- b) record linkage ✸ **X**
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

1996 – CHRISANDERSON – TOTAL: 7 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸
- b) study controls for any additional factor (ALL factors) ✸
  - Reports pre-NST management and NST composition — n
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) — y
  - Reports setting (ICU or non-ICU) — n
  - Reports demographics (age, gender) — y

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸
- b) record linkage ✸ **X**
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

1997 – PNG – TOTAL: 9 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ ____
- b) somewhat representative of the average patient on PN in the community ✸ **X**
- c) selected group of users eg nurses, volunteers ____
- d) no description of the derivation of the cohort ____

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source ____
- c) no description of the derivation of the non exposed cohort ____

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸ ____
- c) written self report ____
- d) no description ____

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no ____

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸ **X**
- b) study controls for any additional factor (ALL factors) ✸ **X**
  - Reports pre-NST management and NST composition y
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) y
  - Reports setting (ICU or non-ICU) y
  - Reports demographics (age, gender) y

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸ **X**
- b) record linkage ✸ ____
- c) self report ____
- d) no description ____

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no ____

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸ ____
- c) follow up rate < 95 % (select an adequate %) and no description of those lost ____
- d) no statement ____

1999 – TRUJILLO – TOTAL: 7 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸
- b) study controls for any additional factor (ALL factors) ✸
  - Reports pre-NST management and NST composition y
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) y
  - Reports setting (ICU or non-ICU) y
  - Reports demographics (age, gender) n

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸
- b) record linkage ✸ **X**
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

2000 – FETTES – TOTAL: 6 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸
- b) drawn from a different source **X**
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸
- b) study controls for any additional factor (ALL factors) ✸
  - Reports pre-NST management and NST composition y
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) y
  - Reports setting (ICU or non-ICU) n
  - Reports demographics (age, gender) y

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸
- b) record linkage ✸ **X**
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

2004 – SAALWACHTER – TOTAL: 7 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ ____
- b) somewhat representative of the average patient on PN in the community ✸ **X**
- c) selected group of users eg nurses, volunteers ____
- d) no description of the derivation of the cohort ____

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source ____
- c) no description of the derivation of the non exposed cohort ____

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸ ____
- c) written self report ____
- d) no description ____

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no ____

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸ ____
- b) study controls for any additional factor (ALL factors) ✸ ____
  - Reports pre-NST management and NST composition — y
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) — n
  - Reports setting (ICU or non-ICU) — n
  - Reports demographics (age, gender) — n

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸ ____
- b) record linkage ✸ **X**
- c) self report ____
- d) no description ____

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no ____

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸ ____
- c) follow up rate < 95 % (select an adequate %) and no description of those lost ____
- d) no statement ____

2005 – KENNEDY – TOTAL: 7 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸
- b) study controls for any additional factor (ALL factors) ✸
  - Reports pre-NST management and NST composition y
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) y
  - Reports setting (ICU or non-ICU) n
  - Reports demographics (age, gender) y

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸ **X**
- b) record linkage ✸
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

2007 – HEARNSHAW – TOTAL: 7 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸
- b) study controls for any additional factor (ALL factors) ✸
  - Reports pre-NST management and NST composition — n
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) — y
  - Reports setting (ICU or non-ICU) — y
  - Reports demographics (age, gender) — y

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸
- b) record linkage ✸ **X**
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

2010 – WALSHE – TOTAL: 8 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸ **X**
- b) study controls for any additional factor (ALL factors) ✸
  - Reports pre-NST management and NST composition — n
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) — y
  - Reports setting (ICU or non-ICU) — n
  - Reports demographics (age, gender) — y

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸
- b) record linkage ✸ **X**
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

2010 – SRIRAM – TOTAL: 7 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸
- b) study controls for any additional factor (ALL factors) ✸
  - Reports pre-NST management and NST composition — y
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) — n
  - Reports setting (ICU or non-ICU) — y
  - Reports demographics (age, gender) — y

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸
- b) record linkage ✸ **X**
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

2010 – BOITANO – TOTAL: 7 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ ____
- b) somewhat representative of the average patient on PN in the community ✸ **X**
- c) selected group of users eg nurses, volunteers ____
- d) no description of the derivation of the cohort ____

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source ____
- c) no description of the derivation of the non exposed cohort ____

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸ ____
- c) written self report ____
- d) no description ____

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no ____

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸ ____
- b) study controls for any additional factor (ALL factors) ✸ ____
  - Reports pre-NST management and NST composition — y
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) — y
  - Reports setting (ICU or non-ICU) — n
  - Reports demographics (age, gender) — n

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸ ____
- b) record linkage ✸ **X**
- c) self report ____
- d) no description ____

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no ____

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸ ____
- c) follow up rate < 95 % (select an adequate %) and no description of those lost ____
- d) no statement ____

2011 – MARTIN – TOTAL: 6 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸
- b) drawn from a different source **X**
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸
- b) study controls for any additional factor (ALL factors) ✸
  - Reports pre-NST management and NST composition n
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) n
  - Reports setting (ICU or non-ICU) n
  - Reports demographics (age, gender) n

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸
- b) record linkage ✸ **X**
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸ **X**
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

2012 – LÓPEZ-MARTÍN – TOTAL: 7 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸
- b) study controls for any additional factor (ALL factors) ✸
  - Reports pre-NST management and NST composition n
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) n
  - Reports setting (ICU or non-ICU) y
  - Reports demographics (age, gender) n

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸
- b) record linkage ✸ **X**
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

2013 – CHONG – TOTAL: 8 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸
- b) study controls for any additional factor (ALL factors) ✸ **X**
  - Reports pre-NST management and NST composition y
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) y
  - Reports setting (ICU or non-ICU) y
  - Reports demographics (age, gender) y

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸
- b) record linkage ✸ **X**
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

2014 – HVAS – TOTAL: 9 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸ **X**
- b) study controls for any additional factor (ALL factors) ✸ **X**
  - Reports pre-NST management and NST composition y
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) y
  - Reports setting (ICU or non-ICU) y
  - Reports demographics (age, gender) y

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸
- b) record linkage ✸ **X**
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

2015 – PARENT – TOTAL: 6 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸
- b) study controls for any additional factor (ALL factors) ✸
  - Reports pre-NST management and NST composition n
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) n
  - Reports setting (ICU or non-ICU) y
  - Reports demographics (age, gender) y

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸
- b) record linkage ✸
- c) self report
- d) no description **X**

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

2016 – PRADO – TOTAL: 7 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸
- b) study controls for any additional factor (ALL factors) ✸
  - - Reports pre-NST management and NST composition n
  - - Reports catheter-related complications (infective, thrombotic, and/or metabolic) y
  - - Reports setting (ICU or non-ICU) y
  - - Reports demographics (age, gender) y

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸
- b) record linkage ✸ **X**
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

2016 – BRAUN – TOTAL: 7 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸
- b) study controls for any additional factor (ALL factors) ✸
  - Reports pre-NST management and NST composition — n
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) — y
  - Reports setting (ICU or non-ICU) — n
  - Reports demographics (age, gender) — y

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸
- b) record linkage ✸ **X**
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement

2018 – LEE – TOTAL: 7 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ ____
- b) somewhat representative of the average patient on PN in the community ✸ **X**
- c) selected group of users eg nurses, volunteers ____
- d) no description of the derivation of the cohort ____

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source ____
- c) no description of the derivation of the non exposed cohort ____

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸ ____
- c) written self report ____
- d) no description ____

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no ____

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸ ____
- b) study controls for any additional factor (ALL factors) ✸ ____
  - Reports pre-NST management and NST composition — y
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) — n
  - Reports setting (ICU or non-ICU) — y
  - Reports demographics (age, gender) — y

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸ ____
- b) record linkage ✸ **X**
- c) self report ____
- d) no description ____

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no ____

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸ ____
- c) follow up rate < 95 % (select an adequate %) and no description of those lost ____
- d) no statement ____

2019 – MEYER – TOTAL: 7 /9

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
- a) truly representative of the average patient on PN in the community ✸ **X**
- b) somewhat representative of the average patient on PN in the community ✸
- c) selected group of users eg nurses, volunteers
- d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
- a) drawn from the same community as the exposed cohort ✸ **X**
- b) drawn from a different source
- c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
- a) secure record (eg surgical records) ✸ **X**
- b) structured interview ✸
- c) written self report
- d) no description

4) Demonstration that outcome of interest was not present at start of study
- a) yes ✸ **X**
- b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
- a) study controls for Definition of catheter-related infection (most important factor) ✸
- b) study controls for any additional factor (ALL factors) ✸
  - Reports pre-NST management and NST composition y
  - Reports catheter-related complications (infective, thrombotic, and/or metabolic) y
  - Reports setting (ICU or non-ICU) n
  - Reports demographics (age, gender) y

Outcome

1) Assessment of outcome
- a) independent blind assessment ✸
- b) record linkage ✸ **X**
- c) self report
- d) no description

2) Was follow-up long enough for outcomes to occur
- a) yes ≥1 week (select an adequate follow up period for outcome of interest) ✸ **X**
- b) no

3) Adequacy of follow up of cohorts
- a) complete follow up - all subjects accounted for ✸ **X**
- b) subjects lost to follow up unlikely to introduce bias - small number lost ≥ 95 % (select an adequate %) follow up, or description provided of those lost) ✸
- c) follow up rate < 95 % (select an adequate %) and no description of those lost
- d) no statement